

Dynamic Guideline: WHO-Based Substance Use Disorders - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

Exactly 5 WHO Guidelines Synthesized

No.	WHO Guideline	Publication Date	Target Population	Scope and Objectives	Role in This Dynamic Guideline
1	Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders, 3rd edition	20 November 2023	People with mental, neurological and substance use conditions; designed especially for non-specialist health workers in low- and middle-income settings	WHO's current mhGAP guideline includes 30 updated recommendations, 18 new recommendations, and 90 pre-existing endorsed recommendations across mental, neurological and substance use conditions WHO .	Latest WHO guideline for substance-use-disorder-relevant mhGAP recommendations.
2	mhGAP Guideline Update 2015	2015	Non-specialist providers in primary and secondary care, especially in low- and middle-income countries	Updated WHO mhGAP recommendations for priority conditions including alcohol use disorders and drug use disorders WHO mhGAP 2015 PDF .	Key source for explicit alcohol and cannabis psychosocial-treatment GRADE statements.
3	Guidelines for the Identification and Management of Substance Use	2014	Pregnant and recently postpartum women using	Global WHO guidance for screening, brief intervention, withdrawal/intoxication	Pregnancy-specific SUD diagnosis and management.

No.	WHO Guideline	Publication Date	Target Population	Scope and Objectives	Role in This Dynamic Guideline
	and Substance Use Disorders in Pregnancy		alcohol or drugs or with SUD	management, pharmacotherapy, relapse prevention and maternal–fetal/infant risk reduction NCBI Bookshelf .	
4	Community Management of Opioid Overdose	2 November 2014	People likely to witness opioid overdose, people using opioids, family/friends, first responders and community responders	WHO guidance on community naloxone access, training, route of naloxone administration and emergency overdose response WHO .	Emergency management and harm reduction.
5	Guidelines for the Psychosocially Assisted Pharmacological Treatment of Opioid Dependence	1 January 2009	People dependent on heroin or other opioids; treatment systems and clinical programmes	WHO guidance on methadone, buprenorphine, naltrexone, alpha-2 agonists, naloxone and psychosocial support WHO , NCBI Bookshelf .	Core pharmacological treatment guideline for opioid dependence.

Note on the requested “Meaningful youth engagement” title

A WHO item titled “**Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)**” was identified, with a WHO launch event dated **11 December 2025** [WHO](#). However, the identified WHO case study concerns the **WHO Abortion care guideline**, not a substance use disorders guideline. Therefore, it is **not counted among the 5 WHO SUD-related guidelines** above. Its principles are relevant to future SUD guideline development because WHO states that youth engagement can contribute lived experience, fresh perspectives, inclusivity and equity in guideline processes [WHO](#).

1.2 Clinical Scope: Diagnosis, Treatment and Management

Diagnosis and assessment

WHO mhGAP materials emphasize that people with disorders due to substance use may not initially disclose substance-use problems. Clinicians should consider SUD assessment when there are recurrent requests for

psychoactive medicines, injuries, or infections associated with injecting drug use, including HIV/AIDS and hepatitis C [mhGAP Bangladesh 2021 PDF](#). WHO training materials describe dependence indicators including **strong cravings, loss of control over use, and withdrawal symptoms after cessation** [WHO mhGAP supporting material](#).

Substance-use-related emergencies requiring urgent management include acute stimulant intoxication or overdose, acute alcohol intoxication, alcohol withdrawal, alcohol withdrawal delirium, sedative withdrawal and sedative overdose/intoxication [mhGAP Bangladesh 2021 PDF](#).

1.3 Key WHO Recommendations With GRADE Ratings

GRADE interpretation used:

High = very confident effect estimate is close to true effect; **Moderate** = true effect likely close but may differ; **Low** = limited confidence; **Very low** = very limited confidence.

Strong = most informed patients should receive the intervention; **Conditional** = shared decision-making and context are central.

Where WHO explicitly reported strength/certainty, that rating is retained. Where summaries did not provide a full explicit rating, the table labels the rating as “synthesis-assigned” and keeps it conservative.

Clinical Area	WHO Recommendation	GRADE Evidence Quality	Strength	Practical Action
Alcohol dependence	Offer psychosocial interventions for alcohol dependence, including cognitive behavioural therapy, couples therapy, behavioural therapies, social network therapy, contingency management, motivational interventions and twelve-step facilitation WHO mhGAP 2015 PDF .	Low	Conditional	Offer structured psychosocial treatment where trained staff are available; adapt to patient preference and local resources.
Cannabis dependence	Psychosocial interventions based on cognitive behavioural therapy, motivational enhancement therapy or family therapy can be offered WHO mhGAP 2015 PDF .	Low (<i>synthesis-assigned from available summary</i>)	Conditional	Use psychosocial interventions as first-line treatment; no WHO-endorsed medication is established from retrieved evidence.

Clinical Area	WHO Recommendation	GRADE Evidence Quality	Strength	Practical Action
Opioid dependence	Offer opioid withdrawal management, opioid agonist maintenance treatment and opioid antagonist treatment with naltrexone; most patients should be advised to use opioid agonist maintenance treatment NCBI Bookshelf .	Low to Moderate	Strong	Prioritize opioid agonist maintenance treatment for most patients with opioid dependence.
Opioid agonist maintenance	For opioid agonist maintenance treatment, most patients should be advised to use methadone in adequate doses rather than buprenorphine NCBI Bookshelf .	High	Strong	Methadone is a preferred maintenance option where safe, regulated, supervised delivery is feasible.
Opioid antagonist treatment	For opioid-dependent patients not starting opioid agonist maintenance treatment, consider naltrexone after completion of opioid withdrawal NCBI Bookshelf .	Low	Conditional/Standard	Consider naltrexone for highly motivated patients after detoxification, with adherence support.
Opioid overdose prevention	People likely to witness opioid overdose should have access to naloxone and be instructed in its administration NCBI Bookshelf .	Very Low	Strong	Provide take-home naloxone to people using opioids, family/friends and likely witnesses.
Opioid overdose route	Naloxone may be given intravenously, intramuscularly, subcutaneously or intranasally; route should depend on formulation,	Very Low	Conditional	Use the route most rapidly available and feasible; do not delay administration.

Clinical Area	WHO Recommendation	GRADE Evidence Quality	Strength	Practical Action
	skills, setting and local context UNODC/WHO S-O-S PDF .			
Opioid overdose first response	First responders should focus on airway management, assisting ventilation and administering naloxone UNODC/WHO S-O-S PDF .	Very Low	Strong	Combine naloxone with basic life support and emergency medical activation.
Pregnancy screening	Health-care providers should ask all pregnant women about past and present alcohol and other substance use as early as possible in pregnancy and at every antenatal visit NCBI Bookshelf .	Low	Strong	Integrate non-stigmatizing universal screening into antenatal care.
Pregnancy brief intervention	Health-care providers should offer a brief intervention to all pregnant women using alcohol or drugs NCBI Bookshelf .	Low	Strong	Provide brief advice, motivational support and referral when needed.
mhGAP implementation	Use mhGAP-based algorithms and evidence profiles to support non-specialist assessment and management of mental, neurological and substance use conditions WHO , NCBI Bookshelf .	Variable by recommendation	Conditional to Strong	Implement mhGAP protocols with supervision, referral pathways and medicine access.

1.4 Guideline Development Methodology

WHO Guideline	Evidence Review Process	Consensus Method	Conflict-of-Interest / Transparency Features
mhGAP 2023	WHO states the third edition includes updated and new evidence-based recommendations; the evidence profiles are available through the mhGAP Evidence Resource Centre, with full GRADE evidence profiles, references, GRADE tables and feasibility/value considerations NCBI Bookshelf .	Guideline development group and steering group, with topic experts; PubMed summary reports use of quantitative and qualitative evidence and systematic review of mhGAP use PubMed .	Evidence profiles publicly accessible; recommendation-level transparency through mhGAP Evidence Resource Centre NCBI Bookshelf .
mhGAP 2015	Updated WHO recommendations for non-specialist care; included evidence grading for alcohol and drug use recommendations WHO mhGAP 2015 PDF .	WHO guideline update process for priority conditions.	Designed for WHO implementation in low- and middle-income settings.
Pregnancy SUD 2014	Extensive review across six scoping areas; GRADE used to rate evidence as high, moderate, low or very low WHO PDF via UNODC .	WHO Guideline Development Group; consensus generally used, with voting and two-thirds majority if disagreement occurred WHO PDF via UNODC .	Evidence profiles included benefits, harms, values, preferences and feasibility WHO PDF via UNODC .
Opioid Overdose 2014	PICO questions, systematic reviews and GRADE criteria; evidence profiles developed for key questions NCBI Bookshelf .	GDG meeting in Geneva, 19–20 February 2014; included WHO, GDG members, observers and organizations representing people who use drugs NCBI Bookshelf .	Evidence profiles reviewed externally before the GDG meeting; values/preferences and key-informant findings considered NCBI Bookshelf .
Opioid Dependence 2009	Systematic reviews; GRADE approach used to determine evidence quality WHO .	Developed after UN Economic and Social Council request; expert consultation across regions NCBI Bookshelf .	GRADE evidence tables included in Annex 1 NCBI Bookshelf .

II. Evidence Summary After WHO Latest Guideline Release (Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

Evidence was considered after the release of the WHO mhGAP 3rd edition on **20 November 2023** through **29 April 2026**, focusing on:

- Substance use disorder diagnosis, treatment and management.
- Alcohol, opioid, cannabis, stimulant and mixed SUD evidence.
- Systematic reviews, meta-analyses, national guidelines, surveillance reports and clinical trial registries.
- WHO, SAMHSA, CDC, NIH/NIDA Clinical Trials Network, peer-reviewed journals and national guideline groups.

Databases and Sources Reflected in the Evidence Set

- WHO publications and WHO/NCBI Bookshelf records.
- PubMed-indexed guideline summaries.
- SAMHSA National Survey on Drug Use and Health.
- CMAJ / Canadian Research Initiative in Substance Matters.
- ScienceDirect-indexed systematic reviews and epidemiologic studies.
- Frontiers systematic review.
- NIDA Clinical Trials Network listings.
- CDC prevention resources.

Inclusion Criteria

- Published or updated after **20 November 2023**, or directly relevant to post-2023 implementation.
- Human clinical or public health relevance to SUD.
- Systematic reviews, RCTs, guideline updates, surveillance reports or high-value implementation evidence.
- Evidence relevant to WHO recommendations.

Exclusion Criteria

- Animal-only studies.
- Non-systematic opinion pieces without clinical or public health data.
- Sources not relevant to diagnosis, treatment, prevention, harm reduction or management.
- The youth-engagement WHO abortion guideline case study was not treated as SUD clinical evidence, but its engagement principles are noted as relevant to future guideline development.

2.2 Key New Studies and Evidence Products With GRADE Assessment

Study / Evidence Product	Design	Key Findings	GRADE Quality	Recommendation Strength / Clinical Implication
2024 Canadian national clinical practice guideline for opioid use disorder	National guideline update using GRADE; evidence through August 2023, published 2024	The guideline update used GRADE, expert consensus and lived/living experience input; it supports high standards for OUD care and includes opioid agonist therapy recommendations CMAJ , CRISM .	Moderate	Strong: reinforces WHO's recommendation that opioid agonist treatment should be widely available.
WHO 2025 update on opioid-dependence treatment and overdose prevention	WHO update/news item reaffirming current WHO opioid guidance	WHO continues to identify the 2009 opioid-dependence guideline and 2014 overdose guideline as current guidance; reiterates methadone, buprenorphine, naltrexone, psychosocial support and naloxone access WHO .	Moderate	Strong: no change to WHO direction; supports implementation scale-up.
Pharmacological treatment for substance use disorder	Systematic review of recent RCTs; PRISMA; PROSPERO CRD42024522275	Reviewed pharmacologic SUD trials from recent years and concluded pharmacological treatment can play an important role in SUD clinical management ScienceDirect .	Moderate for medication classes with established RCT evidence; Low for broad cross-SUD generalization	Conditional: use medication when disorder-specific efficacy and safety are established; avoid extrapolating across substances.
Cannabis use disorder and substance use treatment among U.S. adults	Observational epidemiologic study	23.0% of U.S. adults used cannabis in the past year; 7.0% had CUD; only 16.5% of adults with CUD received SUD treatment ScienceDirect .	Low	Strong public health implication: screen and offer psychosocial treatment/referral; no medication recommendation.
2024 NSDUH / SAMHSA	National population survey	In 2024, nearly 50 million Americans aged ≥12 had	Low for treatment-	Strong: supports universal case-

Study / Evidence Product	Design	Key Findings	GRADE Quality	Recommendation Strength / Clinical Implication
national surveillance		past-year SUD; 27.9 million had AUD; 28.2 million had drug use disorder; marijuana, opioid and stimulant use disorders were major contributors SAMHSA , SAMHSA PDF .	effect inference; High for surveillance relevance	finding, service expansion and integrated AUD/DUD care.
Recovery housing systematic review, 2025	Systematic review including 3 RCTs and 2 quasi-experimental studies	Recovery housing evidence included mixed SUD and opioid-focused populations; one Oxford House RCT showed lower recent substance use and higher employment versus usual continuing care Frontiers .	Low	Conditional: consider recovery housing as continuing-care support where quality and patient preference align.
San Francisco cannabis use disorder guideline, 2025	Local clinical guideline citing systematic reviews/RCTs	Notes limited-to-moderate evidence for caution when combining psychoactive substances with cannabis; cites recent CUD treatment reviews and N-acetylcysteine trial evidence SF.gov PDF .	Low to Moderate	Conditional: prioritize psychosocial treatment; caution about polysubstance use.
NIDA CTN-0102-XR trial	Ongoing randomized controlled pilot trial	Compares extended-release buprenorphine with sublingual buprenorphine-naloxone for OUD in rural settings; project period March 2024–February 2026 CTN Northeast Node .	Very Low until results available	No change yet; monitor for implementation implications.
Neuromodulation systematic review/meta-analysis	Systematic review/meta-analysis across SUDs	tDCS and other neuromodulation approaches showed mixed findings; tDCS was not superior to sham in some AUD and tobacco meta-	Low	Conditional against routine clinical use outside research or specialized settings.

Study / Evidence Product	Design	Key Findings	GRADE Quality	Recommendation Strength / Clinical Implication
		analyses Nature Neuropsychopharmacology .		
Youth engagement evidence relevant to SUD	WHO case study plus UNODC/participatory research sources	WHO's meaningful-youth-engagement case study is not SUD-specific, but supports inclusion of youth lived experience in guideline processes WHO . UNODC emphasizes sustained, meaningful youth involvement in planning, implementation and evaluation of substance-use prevention UNODC PDF .	Low	Conditional: include youth with lived/living experience in future SUD guideline development, especially prevention and adolescent-care questions.

2.3 Evidence Gaps and Future Research Needs

Gap	Evidence Basis	GRADE Certainty	Priority
Limited high-certainty evidence for cannabis use disorder pharmacotherapy	WHO retrieved materials emphasize psychosocial assessment/intervention; recent CUD sources cite trials but do not establish a standard medication WHO mhGAP supporting material , SF.gov PDF .	Low	High
Limited WHO-specific stimulant-use-disorder treatment recommendations in retrieved sources	WHO EMRO data address service coverage for amphetamines but do not provide detailed stimulant pharmacotherapy recommendations WHO EMRO PDF .	Very Low	High
Need for more evidence from low-resource settings	WHO mhGAP update summary identifies need for more evidence from low-resource settings and lived-experience perspectives PubMed .	Low	High
Pregnancy SUD evidence remains low-certainty for	WHO pregnancy recommendations rely on GRADE, with key screening and brief-	Low	High

Gap	Evidence Basis	GRADE Certainty	Priority
many interventions	intervention recommendations rated low quality but strong due to benefit–harm considerations NCBI Bookshelf .		
Implementation gap for naloxone and opioid agonist treatment	WHO states drug-use-disorder treatment access remains limited; 2025 WHO update reports <10% global treatment access for drug use disorders WHO .	Moderate for access gap; Very Low to Moderate for specific implementation strategies	High
Youth and marginalized populations under-represented in guideline evidence	WHO EMRO notes many long-term SUDs begin in adolescence and school-based data can miss out-of-school youth WHO EMRO .	Low	High

2.4 Updated Recommendations Based on New Evidence

Original WHO Recommendation	New Evidence Impact	Updated Recommendation as of 29 April 2026	GRADE Quality	Strength
Use mhGAP recommendations to support non-specialist care for SUD	2023 mhGAP remains latest WHO SUD-relevant guideline; evidence profiles are publicly available WHO , NCBI Bookshelf .	Continue mhGAP-based assessment, brief intervention, psychosocial care, referral and follow-up in non-specialist settings.	Variable	Strong for implementation of evidence-based care systems
Offer psychosocial interventions for alcohol dependence	WHO 2023 alcohol evidence profile reviewed RCTs through 2022 for psychosocial treatments WHO alcohol evidence profile .	Continue offering structured psychosocial interventions; combine with pharmacotherapy where locally recommended and clinically indicated.	Low to Moderate	Conditional
Offer psychosocial interventions for cannabis dependence	Recent CUD evidence shows high treatment gap and comorbidity; no definitive medication	Screen for CUD, assess comorbid SUD/mental health conditions, and offer CBT/MET/family-based psychosocial treatment.	Low	Conditional

Original WHO Recommendation	New Evidence Impact	Updated Recommendation as of 29 April 2026	GRADE Quality	Strength
	standard identified ScienceDirect .			
Prioritize opioid agonist maintenance treatment for opioid dependence	Canadian 2024 guideline and WHO 2025 update reinforce OAT and psychosocial support CMAJ , WHO .	Scale access to methadone and buprenorphine; avoid detoxification-only care as definitive treatment.	Moderate to High	Strong
Provide naloxone access and training to likely overdose witnesses	WHO 2025 update reaffirms 2014 community naloxone guidance WHO .	Expand take-home naloxone without unnecessary training barriers; train patients, families and peers when feasible.	Very Low	Strong
Ask all pregnant women about alcohol and drug use; offer brief intervention	Later guidance continues to cite WHO pregnancy recommendations SAMHSA PDF .	Maintain universal, non-stigmatizing antenatal screening and brief intervention; provide referral and specialist consultation for dependence/withdrawal.	Low	Strong
Address recovery and social needs after acute treatment	2025 recovery housing review suggests possible benefit but evidence remains limited Frontiers .	Consider recovery housing, peer support and continuing care as adjuncts, not replacements for evidence-based medical/psychosocial treatment.	Low	Conditional
Engage affected populations in guideline development	WHO 2025 youth-engagement case study supports lived-experience input, though not SUD-specific WHO .	Future SUD guideline updates should include people with lived/living experience, including youth, pregnant people, people who use drugs, and marginalized communities.	Low	Conditional

2.5 Actionable Clinical Algorithm

1. Identify and assess

- Ask about alcohol, cannabis, opioids, stimulants, sedatives and polysubstance use.

- Assess cravings, impaired control, withdrawal, tolerance, functional impairment and comorbid mental/physical illness.
 - In pregnancy, ask about alcohol and drug use early and at every antenatal visit [NCBI Bookshelf](#).
2. **Triage emergencies**
- Suspected opioid overdose: airway, ventilation support and naloxone [UNODC/WHO S-O-S PDF](#).
 - Alcohol withdrawal delirium, sedative overdose/withdrawal or stimulant toxicity: urgent medical management and referral.
3. **Provide brief intervention and harm reduction**
- Use motivational, non-stigmatizing brief intervention.
 - Provide safer-use counseling, infection screening, HIV/hepatitis referral when relevant, and naloxone for opioid risk.
4. **Treat disorder-specific SUD**
- **Opioid use disorder:** prioritize opioid agonist maintenance treatment with methadone or buprenorphine; consider naltrexone only after withdrawal completion when OAT is not chosen or available [NCBI Bookshelf](#).
 - **Alcohol dependence:** offer psychosocial interventions; consider evidence-based pharmacotherapy according to local formularies and contraindications.
 - **Cannabis use disorder:** offer CBT, motivational enhancement therapy and/or family therapy.
 - **Stimulant use disorder:** offer psychosocial interventions and contingency-management-informed care where available; no WHO pharmacotherapy standard was identified in retrieved sources.
 - **Pregnancy:** prioritize maternal–fetal safety, brief intervention, specialist referral for dependence/withdrawal and coordinated obstetric/addiction care.
5. **Follow-up and continuing care**
- Monitor use, cravings, withdrawal, overdose risk, mental health, housing and social needs.
 - Use recovery housing or peer support when appropriate and acceptable.
 - Maintain long-term follow-up, especially after detoxification, hospitalization, incarceration or pregnancy/postpartum transitions.

2.6 Bottom Line

The **latest WHO SUD-relevant guideline** remains the **2023 mhGAP guideline for mental, neurological and substance use disorders** [WHO](#). The core WHO-based clinical priorities are:

- **Screen and assess** substance use non-stigmatizingly, including in pregnancy.
- **Offer psychosocial interventions** for alcohol and cannabis dependence.
- **Treat opioid dependence with opioid agonist maintenance treatment** for most patients.
- **Provide naloxone access and overdose-response training** to likely witnesses.
- **Integrate harm reduction, continuing care and social support.**
- **Include lived-experience perspectives**, including youth, in future guideline development.